



February 23, 2026

The Honorable Robert F. Kennedy, Jr., JD, LL.M.
Secretary
U.S. Department of Health and Human Services
200 Independence Avenue SW
Washington, DC 20201

RE: HHS Health Sector AI RFI - Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care; RIN 0955-AA13

Dear Secretary Kennedy:

On behalf of the American Podiatric Medical Association (APMA), the premier professional organization representing the vast majority of the nation's doctors of podiatric medicine, also known as podiatrists or podiatric physicians and surgeons, I thank you for the opportunity to respond to the Department of Health and Human Services' (HHS) "Health Sector AI RFI - Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care."

APMA applauds HHS' proactive approach and embrace of artificial intelligence (AI) in advancing its ultimate goals of improved patient care and clinical outcomes while reducing clinician burden. However, we respectfully urge HHS and its affiliated agencies to prioritize policies that integrate AI technology to supplement, not supplant, the work of physicians. HHS must proceed thoughtfully and deliberately, ensuring that the perspectives of all stakeholders are carefully evaluated and meaningfully incorporated before moving forward too quickly.

Specifically, APMA urges HHS to ensure the following in any application or regulation of AI in health care decisions, claims review, and other related processes:

1. Transparency and human oversight in coverage decisions;
2. Establishment of interoperability standards for AI in health care settings so that assistive AI reduces physician administration burden, versus adding to it; and
3. Development of advisory bodies that provide opportunity for physician engagement and input so that all perspectives, including those of podiatric physicians, are represented in the development process for all AI regulatory and sub-regulatory guidance.

Transparency and Human Oversight in Coverage Decisions

While the use of AI-driven algorithms is becoming increasingly common in prior authorization (PA) models, federal guardrails ensuring clinically accurate claim determinations have failed to keep pace. Plans and payers largely omit public data around their processes that are often vague or opaque, leaving

major concerns from physicians. Additionally, AI algorithms regularly rely on generalized data that oversimplifies patient encounters and leads to wrongful denials of care. Training data may include biased samples, which can limit their applicability to relevant populations and inappropriately restrict access to care. Furthermore, AI-driven algorithms can automatically downcode evaluation and management services, leading to reduced reimbursement for podiatric physicians.

Given these concerns, APMA urges HHS to promote policies that strengthen transparency and ensure sufficient guardrails around the use of AI in coverage decisions. Specifically, APMA recommends that HHS mandate or ensure the following:

- Prioritization of patient-centered data and physician judgment in AI-driven PA models
 - o APMA underscores the need for direct physician oversight and collaboration on AI models and technology that impact patient care and the PA process. It is equally important to ensure human oversight in AI-supported coverage decisions. AI-based denials should be reviewed by qualified physicians who have the clinical training needed to inform decisions regarding medical necessity.
- Federal reporting requirements to ensure transparency in data and processes for coverage decisions with PA using AI, including public access to algorithms and explanations of the data used to train the AI. The reporting requirements should also detail the percentage of claims determined by AI, including approval and denial rates.
- Strict enforcement measures to ensure AI-driven models use individualized clinical records rather than broad data sets to determine claim denial or approval.
- Disclosure to both providers and patients when AI is part of the PA decision.

Physician Representation in Emerging AI Regulation

Physicians are one of the primary stakeholders impacted by the unregulated use of AI, including in payer claims decisions. Including their perspective is essential in ensuring effective and ethical design, deployment, and regulation of AI use in healthcare settings. With respect to regulations and federal oversight, perspectives from physicians will help create regulations that are ethically grounded and clinically sound and help to maximize the likelihood that AI serves as a tool to enhance, rather than hinder access to care. To help achieve this goal, HHS should create health care-related AI advisory bodies which include podiatric physicians, to better inform any policy proposals or recommendations.

Assistive AI's Role in Reducing Administrative Burden

Lastly, APMA agrees that there is opportunity for increased efficiencies and administrative burden reduction in clinical practice when physicians incorporate AI. When used as a supportive tool¹, assistive AI reduces burden with patient documentation and visit summaries, and creates opportunity for physicians to provide more direct, patient-centered care. Assistive AI also presents various opportunities to strengthen health record interoperability by helping to standardize, interpret, and connect disparate data sources, which in turn can bolster care coordination to improve patient outcome.

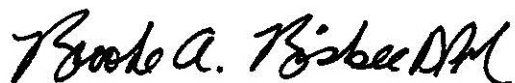
¹ APMA is in alignment with the idea of “augmented intelligence”, a [conceptualization](#) of artificial intelligence put forth by the American Medical Association that focuses on AI’s assistive role, emphasizing that its design enhances human intelligence rather than replaces it.

APMA supports development of AI interoperability standards in healthcare. Standardization promotes secure data exchange, reduces fragmentation, and enables seamless communication between electronic health records (EHRs), diagnostic platforms, and clinical tools. Equally important are clear safeguards that preserve physician autonomy in clinical decision-making. AI systems should function as supportive tools, with providers remaining the primary and final decision-makers in their patients' care.

Conclusion

Thank you again for the opportunity to respond to this request for information. If you require additional information, please contact Gail Reese, APMA Director Health Policy and Practice, at greese@apma.org or 301-581-9230. Thank you for your time and consideration.

Sincerely,

A handwritten signature in black ink that reads "Brooke A. Bisbee DPM". The signature is fluid and cursive, with the first name "Brooke" and last name "Bisbee" being more prominent than the middle initial "A." and the credential "DPM".

Brooke A. Bisbee, DPM
President